



Health and Social Care Scrutiny Commission

MINUTES of the OPEN section of the Health and Social Care Scrutiny Commission held on Tuesday 27 January 2026 at 7.00 pm at 160, Tooley Street, SE1 2QH

PRESENT: Councillor Suzanne Abachor (Chair)
Councillor Esme Dobson

OTHER MEMBERS PRESENT:

OFFICER & Tara Quinn, Head of Parks and Leisure
PARTNER Toni Ainge, Director of Leisure
SUPPORT: Mr Haitham Hamoda , Consultant Gynaecologist
Bethan Jones, GP
Dr Martin QUIRKE, Senior Architect (DSDC), Lecturer in Design for Ageing and Dementia, University of Stirling
Julie Timbrell, Project Manager, Scrutiny

1. APOLOGIES

Apologies were received from the following councillors:

- Cllr Sandra Rhule
- Cllr Naima Ali
- Cllr Charlie Smith
- Cllr Maria Linforth-Hall

2. NOTIFICATION OF ANY ITEMS OF BUSINESS WHICH THE CHAIR DEEMS URGENT

There were none.

3. DISCLOSURE OF INTERESTS AND DISPENSATIONS

There were none.

4. MINUTES

The minutes of the meeting held on 1 December 2025 were agreed as an accurate record.

5. MENOPAUSE

The chair welcomed the presenters:

- **Mr Haitham Hamoda , Consultant Gynaecologist**, Subspecialist in Reproductive Medicine and Surgery, Clinical Lead Menopause Service, King's College Hospital
- **Bethan Jones, GP** - Blackfriars Medical Practice , Clinical and Care Professional Lead for Women's and Girls' Health - South East London ICB, Clinical Lead for LARC - Southwark Council

Haitham Ahmed, consultant gynaecologist, provided a comprehensive overview of menopause, including its clinical background, symptom variability, risks and benefits of hormone replacement therapy (HRT), and the importance of individualised care, followed by Dr Beth Jones who gave an overview of local practice. There was then a discussion with Commission members. The following was covered:

- **Menopause Definition and Prevalence:** Haitham Ahmed explained that menopause marks the end of the reproductive life cycle, typically occurring between ages 45 and 55, but with significant numbers experiencing it earlier or later. Early menopause can have notable impacts on bone, cardiovascular, and cognitive health, as well as quality of life.
- **Symptom Variability and Duration:** The discussion highlighted that while three out of four women experience menopause symptoms, one in four do not, and symptoms can range from mild to severe. Symptoms often last over seven years, with a third of women experiencing them into their 60s and beyond, underscoring the need for individualised management.
- **Symptom Types and Counselling:** Haitham Ahmed emphasised that symptoms extend beyond hot flushes and night sweats to include anxiety, mood changes, and cognitive symptoms like brain fog. He stressed the importance of comprehensive counselling to address the full spectrum of symptoms and their varied effects.
- **HRT Indications, Benefits, and Risks:** The main indication for HRT is symptom management, but it is also used for premature ovarian

insufficiency and osteoporosis. Haitham Ahmed clarified that while HRT offers significant benefits, including reduced osteoporosis and cardiovascular risk, the associated breast cancer risk is small and should be contextualised alongside other lifestyle risks.

- **Individualised Care and Information Access:** The speakers stressed the importance of guiding women to accurate information, managing expectations, and considering both HRT and non-HRT options. Individualisation of treatment, including dose and duration, is essential, and the benefits of HRT generally outweigh the risks for most women.
- **Training, Education, and Local Initiatives in Menopause Care:** Haitham Ahmed and Dr Beth Jones described the British Menopause Society's (BMS) resources, training programmes, and the development of local and regional initiatives in South East London and Southwark to improve menopause care and reduce inequalities.
- **BMS Resources and Training Programmes:** Haitham Ahmed outlined the BMS's toolkit, which includes videos, consensus statements, and fact sheets for both healthcare professionals and the public. The Principles of Practice of Menopause Care (PPMC) and the Management of Menopause Certificate provide comprehensive, accessible training, with over 2,000 professionals enrolled in the online programme.
- **Local Guidelines and Networks:** The South East London Menopause Network has developed practical guidelines for GPs, streamlined access to medications, and established patient and public support groups. The network aims for at least one GP with a menopause special interest in each practice, supported by council or network funding for training.
- **Primary Care Hubs and Virtual Appointments:** Haitham Ahmed advocated for the creation of primary care hubs, potentially starting with a few sessions per week, to provide local, specialist menopause support and reduce waiting times. The pandemic demonstrated the feasibility of virtual clinics, which could be expanded to improve access.
- **National and Local Data on HRT Prescribing:** Dr Beth Jones presented data showing a 22% national increase in HRT prescriptions and highlighted inequalities, with less deprived areas having higher prescribing rates. In Southwark, HRT prescribing is relatively high compared to similar areas, but there is still room for improvement, especially in addressing deprivation-related disparities.
- **Barriers and Facilitators to Service Development:** Barriers include

funding constraints and disjointed budgets, such as reimbursement issues for contraceptive devices used as HRT. Facilitators include high clinician motivation and the development of local training courses and resources, such as the SEL primary care menopause course and regular network meetings.

6. NURSING CARE HOME SPACE STANDARDS

The chair welcomed **Dr Martin QUIRKE, Senior Architect (DSDC)**, Lecturer in Design for Ageing and Dementia, University of Stirling, who attended virtually.

Dr Martin Quirke presented the principles of dementia-friendly design for care homes, focusing on spatial configuration, visual access, and the importance of supporting autonomy, daily life, and outdoor access, with questions and examples discussed with the commission.

- **Principles of Dementia Design:** Dr Martin Quirke explained that effective dementia design goes beyond minimum room sizes, focusing on spatial relationships, visual access, sensory environment, and support for everyday activities. He referenced frameworks by Fleming and Bennett and tools like Plan-EAT for assessing design quality.
- **Visual Access and Layout:** The presentation highlighted the importance of visual access between spaces, such as being able to see toilets or gardens from key locations, to support independence and reduce staff workload. Examples from the Netherlands and Japan illustrated strengths and trade-offs in different layouts.
- **Outdoor Access and Risk Management:** A member raised concerns about dead-end corridors and limited outdoor access, especially for residents who walk frequently. Dr Quirke emphasised that design solutions, such as courtyards and passive surveillance, can facilitate safe outdoor access and improve well-being, while organisational attitudes to risk also play a role.
- **Nature, sunlight and human touch.** A member highlighted that being outside can be restorative, but unfortunately often the most incapacitated are on the top floor, whereas often the mobile are on the bottom. In addition beds become single, when physical contact is so important. Dr Quirke agreed that sunlight and outdoor space is important to health & wellbeing, and unfortunately this is often not given as much priority as medication and supplements when the evidence shows how beneficial it can be. Appleby Blue is a good

example , however , of good design.

- **Design for Younger Residents and Social Interaction:** The discussion addressed challenges in designing for younger people with dementia, who may have different needs and activity levels. The importance of social interaction, access to fresh air and sufficient space to exercise was emphasised with examples of how design and care practices, such as facilitating engagement with a local running club, can support these needs. Excessive walking can be a sign of restlessness so keeping people occupied is also important
- **Role of Local Authorities and Future Projects:** Dr Quirke advised that local authorities should ask critical questions early in the design process, focusing on how layouts support autonomy, daily life, and staff workload. The Asylum Road project and other local developments were discussed as opportunities to apply these principles.

7. ACCESS TO TOILETS - UPDATE

The chair introduced the item by explaining this an officer update on a previous scrutiny review report making recommendations to cabinet to improve Access to Toilets. The recommendations were largely agreed and a report was circulated with the agenda outlining progress to date.

The officers introduced themselves:

- **Tara Quinn, Head of Parks and Leisure** • Environment, Sustainability & Leisure
- **Toni Ainge, Director of Leisure** • Environment, Sustainability and Leisure
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Toni Ainge and Tara Quinn updated the Commission on the council's public toilet action plan, including recent investments, ongoing challenges with anti-social behaviour, and efforts to improve provision, maintenance, and communication, followed by a discussion with members. The following points were made:

- **Action Plan Development and Working Group:** A cross-council working group was formed to implement recommendations from the cabinet response to the scrutiny review, focusing on improving standards, increasing provision, and enhancing communication about public toilets. The group includes representatives from around ten council teams and a resident member for user perspective.
- **Recent Investments and Projects:** Recent and ongoing projects include refurbishments at Peckham Rye Park, East Street Market,

Dulwich Library, and Brandon Library, with new or improved accessible toilets being added. The council ensures these facilities are listed on national and council maps.

- **Challenges: Anti-Social Behaviour and Resource Constraints:** Anti-social behaviour remains a significant challenge, particularly in unstaffed facilities. The council is working with the community safety team and considering robust design solutions. Resource constraints limit the ability to provide a community toilet scheme or a full public toilet strategy.
- **Engagement with Local Businesses and Communication:** Efforts are underway to encourage local businesses to open their toilets to the public, with incentives and community spirit as motivators. Communication improvements include updating online maps, launching a new campaign, and ensuring information reaches digitally excluded residents.
- **Councillors raised concerns about cleanliness, staffing, and the impact of toilet closures on vulnerable residents.** Suggestions included paid toilets, however officers advised that this could be very expensive even if just aiming to break even.
- **Peckham Rye toilet** at the station is remains unconfirmed, and if this will be before of after the barrier, if provided. Members reported that residents were keen to see provision in the new square. There are believed to be concerns about anti social behaviour from Sustainable Growth. Officers offered to get an update.
- **Community toilet scheme** - officers looking at incentives, however this an area which is difficult to resource. Members spoke about not having access to a toilet in supermarket. Officers said there are levers in planning that can be used to require provision for facilities over a certain size.

RESOLVED

- **Peckham Rye Station Public Toilet Provision:** Follow up will be provided to clarify the current status and future potential for public toilet provision in the Station Square redevelopment at Peckham Rye Station, and the possibility of arranging access to station toilets for non-travellers after the station redevelopment is complete if the toilet is located behind the barrier

8. WORK PROGRAMME

Commission members reviewed the upcoming work plan, discussed scheduling additional meetings to cover safeguarding, FGM, and other topics, and considered the feasibility of agenda items for March meetings.

- **Additional meeting in March** The Commission discussed the need for an extra meeting to accommodate the annual interview with the safeguarding board chair, a report on FGM, and to wrap up the final Safeguarding scrutiny review report.
- **Prioritisation and Recommendations:** Members considered deferring some items, such as the Blue Badge scheme, to avoid overloading meetings and to allow for better preparation and engagement with external stakeholders like TFL.